Blood tests: oestradiol

Key points Most tests are every 3 to 4 months in the first year, then about once a year.

Applies to me	Test	Before starting	First year	Each year after
☐	**Oestradiol level (trough, just before the next dose; for patches, just before a new patch)**	Yes	At each review, every 3–4 months; and 6–8 weeks after a dose change	At each review (yearly, or every 6–12 months)
☐	**Total testosterone**	Yes	At each review; under 2 nmol/L if full suppression is wanted	At each review
☐	**Full blood count, kidney function and electrolytes, liver function**	Yes	At each review	At each review
☐	**LH and FSH**	Yes	Not routine. LH may be checked to see whether your hormone levels are high enough to protect your bones (this does not work on cyproterone)	Same
☐	**Prolactin**	Yes	Only if your prescriber chooses to check it, or if you have symptoms such as new headaches, vision changes or a lot of milky nipple discharge	Same
☐	**Lipids and glucose (or HbA1c)**	Yes, especially with heart risk factors	—	Periodically, especially with other risk factors
☐	**Implants only**	Not used to start therapy	Usually every 3 months after the first implant, to plan the next one	Less often; the blood level guides when the next implant is due

Sources: [1][2][3]; LH, prolactin and bone [22]; implants [13].

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Blood tests: spironolactone

Key points Spironolactone needs potassium and kidney tests.

Applies to me	Test	Before starting	First year	Each year after
☐	**Potassium (electrolytes) and kidney function**	Yes	Product information: weekly at the start, then monthly for 3 months, then every 3 months for the rest of the first year. Other guidance: at each review, and especially important with spironolactone	Product information: every 6 months. Other guidance: at each review
☐	**Also tell your prescriber**	If you take ACE inhibitors, ARBs, anti-inflammatories (NSAIDs), potassium supplements or salt substitutes	—	—

Sources: Spiractin and Aldactone CMI [14] (Product information); AusPATH and the position statement [1][2] (Other guidance). Kidney checks matter even more if you also take PrEP [1][11].

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Blood tests: cyproterone and bicalutamide

Key points Cyproterone and bicalutamide need liver tests.

Tests for cyproterone acetate (androgen blocker)

Applies to me	Test	Before starting	First year	Each year after
☐	**Liver function**	Yes	Regularly during treatment, and at once if you have signs of liver trouble (yellow skin or eyes, dark urine)	Regularly
☐	**Full blood count (red cells)**	Yes (part of everyone's first tests)	Regularly	Regularly
☐	**Adrenal function**	—	The product information says it is checked regularly during treatment	As your prescriber decides

Sources: AusPATH baseline tests and liver monitoring [1][22]; Androcur PI s4.4 (liver, adrenocortical function and red cells) [15]. Liver deaths have been reported at doses of 100 mg a day and above, mostly in men treated for prostate cancer; Australian guidance for feminising therapy gives 12.5 mg once a week up to 25 mg a day [15] [22].

Tests for bicalutamide (not first-line)

Applies to me	Test	Before starting	First year	Each year after
☐	**Liver function**	Yes	Regularly	Regularly
☐	**Tell your prescriber at once**	—	New cough, breathlessness or fever (lung inflammation is listed in the product information)	Same

Sources: AusPATH p28 (not first-line; liver and lung toxicity, including deaths; regular liver tests) [16]; Cosudex PI s4.4, interstitial lung disease added August 2026 [17].

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Blood tests: micronised progesterone

Key points No blood test of its own. It is reviewed with your oestradiol.

Applies to me	Test	Before starting	First year	Each year after
☐	**Blood tests**	No test specific to progesterone	Reviewed with your oestradiol	Reviewed with your oestradiol
☐	**Review**	—	A check after 3–6 months to see whether it is helping and whether to continue	At each review
☐	**Mood**	Discussed	Watched, because low mood is a possible side effect	Watched

Sources: AusPATH p56 [22]. The Prometrium product information also advises caution with depression, epilepsy, diabetes, asthma or fluid retention, and says it may raise ciclosporin levels [18].

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Blood tests: testosterone

Key points Testosterone level and blood count at each review. When the sample is taken depends on the form.

Applies to me	Test	Before starting	First year	Each year after
☐	**Total testosterone (trough)**	Yes	At each review, every 3–4 months; target about 10–20 nmol/L	At each review
☐	**Full blood count (haemoglobin and haematocrit)**	Yes	At each review	At each review; above about 0.50, the dose or interval is reviewed
☐	**Lipids and glucose**	Yes	As your prescriber decides	Periodically
☐	**Liver function**	Yes	At each review	At each review

When the testosterone sample is taken depends on the form [1]:

Form	When the sample is taken
Long-acting injection (undecanoate)	**just before the next injection; the dose is not changed until after three injections**
Short-acting injection (enantate, mixed esters)	**midway between injections, or as a peak and a trough**
1% gel (Testogel) or 5% cream	**in the morning, before that day's dose**
2% gel (Testavan)	**2 to 4 hours after applying**

Sources: [1][2][3]. If you take an SGLT-2 inhibitor (such as empagliflozin or dapagliflozin), the blood count is checked more closely [5][6].

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Blood tests: GnRH analogues

Key points Bone health and mood are watched. Some people also need a heart tracing (ECG).

Applies to me	Test	Before starting	First year	Each year after
☐	**Bone health**	A bone density scan is not routine; it may be considered if you have risk factors	Watched, because blockers pause bone strengthening; long-term use is itself a risk factor, so a scan may be considered	Watched; a scan may be considered with long-term use, or if you take a blocker without oestrogen or testosterone
☐	**Heart tracing (ECG) and electrolytes**	If you have a heart rhythm condition, heart failure or take medicines that affect heart rhythm	Considered periodically in those situations	Same
☐	**Mood**	Discussed	At each review	At each review

Sources: bone [2][7][22]; QT and electrolytes, Zoladex PI s4.4/4.5 (same warning in Eligard, Lucrin and Diphereline) [19]; mood changes reported with GnRH agonists [19].

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Blood tests: medicines used alongside

Key points Some medicines used alongside hormones have tests of their own.

Medicine	Tests
Isotretinoin	**Liver function and lipids before starting, monthly, and at the end of the course; mood watched throughout [20]**
Finasteride	**No routine blood test; it lowers PSA results by about half, so tell whoever orders a prostate test [21]**
PrEP	**Kidney function, HIV and STI tests regularly; kidney tests matter more with spironolactone [1][11]**

These are on the Add-on therapies page.

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Blood tests for each medicine: sources

These tables show the tests usually done for each medicine: before starting, in the first year, and after that. They are a guide to what to expect, not a schedule to follow on your own. Your prescriber sets the actual timing, and it can change with your health, other medicines and how you are going [1][2].

In the first year, reviews are usually every three to four months [1] (three-monthly in the 2019 position statement [2]). Once things are stable, Australia's 2026 standards of care suggest a yearly review [1]; the position statement suggests every six to twelve months [2]. The tables use "Each year" for the stable stage, with the position statement range alongside. Every review, whatever the medicine, also covers blood pressure, weight, mood and how the changes are going for you [2][7].

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Blood tests for each medicine: sources

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